

Medication Error and Adverse Drug Event Reporting

Policy Number	POL-PS-003	Owner / Department	Pharmacy / Patient Safety
Effective Date	2023-05-15	Version	4.0
Last Revised	2026-02-28	Next Review	2028-02-28
Approved By	Patient Safety & Quality Committee	Applies To	All staff who prescribe, transcribe, dispense, administer, or monitor medications

1. Purpose

To standardize detection, response, and reporting of medication errors and adverse drug events (ADEs), and to classify events by harm using the NCC MERP index.

2. Definitions

- **Medication error** — any preventable event that may cause or lead to inappropriate medication use or patient harm.
- **Adverse drug event (ADE)** — harm resulting from the use of a medication.
- **High-alert medication** — a drug bearing heightened risk of significant harm when used in error (e.g., insulin, heparin, opioids, concentrated electrolytes, chemotherapy).

3. Harm Classification (NCC MERP Index)

Classify every medication error by the NCC MERP category, which maps to the facility severity scale.

Category	Meaning	Facility severity
A	Circumstances capable of causing error.	Near Miss
B	Error occurred but did not reach the patient.	Near Miss
C	Reached the patient; no harm.	SEV-3
D	Reached the patient; required monitoring to confirm no harm.	SEV-3
E	Temporary harm requiring intervention.	SEV-2
F	Temporary harm requiring or prolonging hospitalization.	SEV-2
G	Permanent harm.	SEV-1
H	Intervention required to sustain life.	SEV-1
I	Contributed to or resulted in death.	SEV-1

4. Immediate Response

IMPORTANT — If an error reaches the patient, assess and stabilize the patient first, notify the ordering provider immediately, and follow any ordered monitoring or reversal. For a high-alert-medication error reaching the patient, notify the provider and Pharmacy without delay and classify at least SEV-2.

1. Assess the patient and provide clinically indicated care.
2. Notify the ordering/attending provider; obtain orders for monitoring, antidote, or reversal as needed.
3. Notify Pharmacy for guidance and to quarantine any implicated product.
4. File an event report per POL-RM-013; classify by NCC MERP category.

5. Notification Timing

- SEV-1 (NCC MERP G–I): notify provider, Nursing Supervisor, and Risk Management immediately and within 1 hour.
- SEV-2 (NCC MERP E–F): notify provider and Risk Management within 4 hours.
- SEV-3 / Near Miss (NCC MERP A–D): event report within 24 hours.

6. Documentation Requirements

- Medication, dose, route, and the node where the error occurred (prescribing, transcribing, dispensing, administering, monitoring).
- Whether the drug is a high-alert medication.
- Whether the error reached the patient and the NCC MERP category.
- Provider notification and any treatment, monitoring, or reversal provided.

7. References

- NCC MERP Index for Categorizing Medication Errors.
- ISMP List of High-Alert Medications in Acute Care Settings.
- Related: POL-RM-013 (severity & reporting).